BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual cancer-assessment plan

These stages are not equal time intervals. Your team provides dates and reviews results. There is no universal blood tumour-marker panel or scan interval in this guide.

Check	Before starting	During treatment	Between visits	Response review	After treatment
Bloods and organ safety	Full blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune disease, transplants, previous immune side effects and all medicines. Confirm the tumour PD-L1 combined positive score; this is a tumour test, not a routine blood test.	Full blood count, kidney and liver tests and glucose before every 3-week cycle. Thyroid tests every 3–6 weeks; cortisol when clinically indicated.	New symptoms may require extra tests before the next planned review.	Safety bloods do not by themselves show whether treatment controls the cancer.	Keep reporting new symptoms after treatment ends. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.
Thyroid and other hormones	Baseline thyroid function and glucose.	Thyroid tests every 3–6 weeks, including between six-week visits when due.	Report marked thirst, unusual fatigue, dizziness, headache or weakness.	Cortisol or other hormone tests when clinically indicated.	Continue prescribed hormone replacement and monitoring when needed.
Head and neck function	Record weight, intake, swallowing, speech and existing mouth or dental problems.	Review pain, intake, weight and support needs.	Report choking, inability to drink, worsening swallowing or new neck symptoms.	Agree examinations and scans with the oncology team.	Continue rehabilitation and symptom support as needed.
Medicines and practical plan	Confirm the exact prescription, treatment aim and actual urgent contacts.	Bring your medicine list, current support plan and treatment record.	Record symptoms and medicines taken; ask before adding a new product.	Confirm any pause, dose change or stop in writing.	Record the final dose and the teams responsible for follow-up.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00455 v16: official regimen

Tests in your plan

Full blood count

WHEN

Before starting and before each treatment cycle

Checks blood cells and helps assess infection, anaemia and bleeding risk.

Kidney and liver tests

WHEN

Before starting and before each cycle

Checks for organ injury, including possible immune inflammation.

Blood glucose

WHEN

Baseline and before each cycle

Checks blood sugar and possible treatment-related diabetes.

Thyroid function

WHEN

Baseline; every 3–6 weeks during treatment

Checks for an underactive or overactive thyroid.

Cortisol

If part of your plan

WHEN

When clinically indicated

Assesses possible adrenal or pituitary problems when symptoms or findings suggest them.

Tumour PD-L1 combined positive score

WHEN

Before choosing treatment

Checks whether the cancer meets this monotherapy indication; CPS is different from the lung tumour proportion score.

Scans and head and neck examination

If part of your plan

WHEN

As prescribed in the individual disease-assessment plan

Assesses the cancer and the response to treatment separately from safety bloods.

THE WHOLE TREATMENT JOURNEY

Pembrolizumab every 3 weeks for head and neck cancer: treatment and checks

First-line treatment of metastatic or unresectable recurrent head and neck squamous cell carcinoma with a tumour PD-L1 combined positive score (CPS) of at least 1. This indication excludes nasopharyngeal cancer and cancer progressing within six months after curatively intended systemic treatment for locoregionally advanced disease.

Pembrolizumab 200 mg is given through a vein over about 30 minutes on Day 1 every 3 weeks. The whole visit includes safety checks, preparation and observation. Treatment continues until progression or unacceptable toxicity. After 24 months of sustained response without progression, the team may consider stopping; stopping at 24 months is not automatic.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response review	After treatment
Bloods and organ safety NCCP	Full blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune disease, transplants, previous immune side effects and all medicines. Confirm the tumour PD-L1 combined positive score; this is a tumour test, not a routine blood test.	Full blood count, kidney and liver tests and glucose before every 3-week cycle. Thyroid tests every 3–6 weeks; cortisol when clinically indicated.	New symptoms may require extra tests before the next planned review.	Safety bloods do not by themselves show whether treatment controls the cancer.	Keep reporting new symptoms after treatment ends. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.
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Care / check	Before starting	During treatment	Between visits	Response review	After treatment
Medicines and practical plan NCCP	Confirm the exact prescription, treatment aim and actual urgent contacts.	Bring your medicine list, current support plan and treatment record.	Record symptoms and medicines taken; ask before adding a new product.	Confirm any pause, dose change or stop in writing.	Record the final dose and the teams responsible for follow-up.

What each check is for

1 Pembrolizumab monotherapy

Every 3 weeks

Pembrolizumab aims to control head and neck cancer that has returned or spread, relieve symptoms and help some people live longer. Benefit and side effects are reviewed for you.

Ask / act: Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-09 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.46